SUBJECTIVE:
Patient presents with acute onset of severe respiratory distress, reporting inability to breathe and stating, "My chest is caved in." The patient expresses significant discomfort and concern regarding their respiratory status. Further history regarding mechanism of injury, onset, or aggravating/alleviating factors is currently unavailable due to the severity of the presenting complaint and immediate need for medical assessment.
OBJECTIVE:
Expected physical exam findings include signs of significant respiratory distress such as tachypnea, retractions, and use of accessory muscles of respiration, potentially with central cyanosis. Inspection of the chest would likely reveal a visible deformity of the chest wall, potentially with paradoxical movement of a segment during respiration, indicative of a flail segment. Palpation may elicit severe tenderness, crepitus, and instability of the affected ribs. Auscultation of the lungs would likely reveal diminished or absent breath sounds over the affected area, and potentially distant heart sounds. Cardiovascular exam may show tachycardia with potential for hypotension depending on the extent of injury.
Vital signs are unavailable at this time.
ASSESSMENT:
Primary diagnosis:
Acute Respiratory Failure likely secondary to Flail Chest. The patient's reported "caved in" chest and inability to breathe strongly suggest severe chest wall trauma disrupting normal respiratory mechanics.
Differential diagnosis:
Pneumothorax (especially tension pneumothorax), Hemothorax, Severe Pulmonary Contusion, Multiple Rib Fractures without flail segment, Cardiac Tamponade (if trauma involves mediastinum). These conditions can also cause acute respiratory distress and may coexist with or mimic flail chest.
PLAN:
Tests:
* Immediate Chest X-ray (CXR) to evaluate for rib fractures, pneumothorax, hemothorax, and pulmonary contusion.
* Computed Tomography (CT) of the Chest for detailed assessment of chest wall injuries, lung parenchyma, and mediastinal structures.
* Arterial Blood Gas (ABG) to assess oxygenation, ventilation, and acid-base status.
* Complete Blood Count (CBC), Basic Metabolic Panel (BMP), and Coagulation Panel.
* Electrocardiogram (ECG) to rule out cardiac contusion or other cardiac involvement.
* Focused Assessment with Sonography for Trauma (FAST) exam if trauma is suspected.
Medications:
* Supplemental Oxygen immediately, aiming for $SpO_2 > 94\%$.
* Intravenous (IV) analgesia (e.g., opioids) for pain control to improve respiratory effort.
* Consider muscle relaxants or sedation if intubation is required.
* Potential for intubation and mechanical ventilation if respiratory failure worsens or is not amenable to conservative management.
Patient advice:
* N/A in an acute, critical presentation. Focus is on immediate life-saving intervention and stabilization.
Follow-up instructions:
* Immediate hospital admission to a high-acuity setting (e.g., Intensive Care Unit or Trauma Bay).
* Urgent consultation with Trauma Surgery and/or Thoracic Surgery.
* Continuous monitoring of respiratory status, vital signs, and oxygen saturation.
* Serial physical exams to monitor for worsening respiratory compromise or tension pneumothorax.